

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

Building Up: A Blueprint for the Alberta Accessibility and Inclusion Act

Part One — What Real Accessibility and Inclusion Actually Look Like

Section 1.3 — Childhood: FSCD and the Developmental Window

The lifecycle frame established in Section 1.2 begins at birth. Of the three modes of disability support, growth is the mode that applies most powerfully in early childhood, and the mode where Alberta's failures produce the most permanent damage. The clinical evidence is not in dispute. Developmental neuroscience has established, across decades of peer-reviewed research, that the human brain has time-bound windows during which specific kinds of intervention are dramatically more effective than the same intervention provided later. Missing the window does not delay the intervention's benefit. It forecloses it. The capacity that intervention would have produced does not become available later. It simply does not develop.

This is the clinical foundation that the Family Support for Children with Disabilities (FSCD) program in Alberta exists to honour. It is also the clinical foundation Alberta is currently ignoring, at scale, through wait times that have made the foundation impossible to honour. The result is documented harm to thousands of Alberta children, and predictable downstream costs the province will pay across those children's adult lifetimes, in every program that has to absorb the needs the growth mode would have prevented.

What Developmental Windows Actually Are

The human brain is not functionally complete at birth. It develops through a documented sequence of neurological processes — synaptic proliferation, experience-dependent pruning, myelination, cortical specialization — that occur on timelines determined by genetic programming and environmental input acting together. During specific periods of early childhood, the brain is in states of heightened plasticity that allow targeted intervention to shape neural architecture in ways that become substantially more difficult, and in some cases impossible, later in life.

The developmental neuroscience literature distinguishes between *sensitive periods*, during which the brain is particularly receptive to specific experiences, and *critical periods*, during which specific experiences are essential for normal development and the absence of those experiences can produce permanent, irreversible changes in brain structure. Both involve heightened neuroplasticity. Critical periods carry higher stakes because the foreclosure is permanent.

The Public Health Agency of Canada, in alignment with international developmental research, identifies the *first thousand days* of life as the foundational critical period for neurodevelopment. This is the period during which the brain's basic architecture for cognition, language, social processing, and emotional regulation is being assembled. The peer-reviewed clinical literature, summarized in *Infants & Young Children* and replicated across multiple journal venues, confirms the first thousand days as the window during which intervention has the greatest potential for impact and during which the absence of

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intervention produces the most consequential foreclosures.

For specific developmental disabilities, the windows are even more sharply defined. The neuroscience of autism early intervention, summarized in journals including the *International Journal of Developmental Neuroscience* and the U.S. National Institute of Child Health and Human Development, indicates that intervention beginning before age three yields substantially greater positive outcomes than intervention beginning after age five. The Early Start Denver Model (ESDM), one of the most studied early autism interventions, has produced documented IQ improvements averaging approximately 18 points in children who received intervention beginning between 18 and 30 months, compared to approximately 4 points in children receiving standard community intervention beginning later. The face-processing neural systems that support social communication develop in early sensitive periods; children who do not engage with faces during the window may not develop the specialized cortical architecture that face processing requires.

For cerebral palsy and other neurodevelopmental disabilities of motor origin, the rehabilitation literature is consistent: intervention during the first year of life, when motor cortex development is at peak plasticity, has greater impact than the same intervention delivered later. For children with prematurity-related developmental risk, intervention in the neonatal period produces better outcomes than intervention deferred until clear deficit is established.

The principle is consistent across disability categories. *Skills beget skills*, as the developmental economist James Heckman and his colleagues at the University of Chicago have summarized. Early skill acquisition raises the productivity of all later skill acquisition. The growth mode is most effective when it begins early, because the foundations it builds become the substrate that all subsequent development is built on.

What the Investment Returns

The economic literature on early childhood intervention is unusually consistent for a field of social policy. James Heckman, Nobel Laureate in Economics, has summarized decades of research demonstrating that high-quality early childhood development programs deliver returns on investment that exceed standard returns on stock market equity. The most-cited figure is approximately 13% return per child per year for high-quality birth-to-five programs serving disadvantaged children. The aggregate return across childhood is estimated at between \$7 and \$13 in long-term economic benefits per dollar invested, with some studies producing estimates as high as \$17 per dollar.

These returns are not theoretical. They are produced through documented mechanisms: increased educational attainment, higher adult earnings, reduced criminal justice system involvement, reduced reliance on adult social services, improved physical and mental health, and reduced public expenditure across the lifespan. The Carolina Abecedarian Project and the Carolina Approach to Responsive Education, the two longest-running randomized controlled trials in early childhood intervention, have followed participants for over four decades. The economic returns have continued to accumulate across the participants' adult lives, producing the cost-benefit ratios summarized above.

For children with developmental disabilities specifically, the return-on-investment case is even stronger. The cost of failing to intervene during the developmental window is borne not only by the individual but by

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every subsequent system that has to absorb the unmet needs — the school system, the adult disability support system, the healthcare system, the criminal justice system, the homelessness response system, and the family caregivers who absorb what the public systems do not.

Heckman's central conclusion, repeated across his published work, is unambiguous: *the highest rate of return in early childhood development comes from investing as early as possible, from birth through age five, in disadvantaged families. Starting at age three or four is too little too late.* For families raising children with developmental disabilities, the urgency is greater still. The window is not metaphorical. It is a neurological reality. Investment delayed beyond the window does not produce the same returns. The returns are not deferred. They are foreclosed.

KEY FINDINGS — DEVELOPMENTAL WINDOWS AND ECONOMIC RETURN

The first thousand days — the foundational critical period for neurodevelopment, during which the brain's basic architecture for cognition, language, social processing, and emotional regulation is assembled. Source: Public Health Agency of Canada; international developmental neuroscience literature.

Autism early intervention — intervention beginning before age three produces substantially greater outcomes than intervention beginning after age five. The Early Start Denver Model has documented IQ improvements averaging approximately 18 points with early intervention versus approximately 4 points with later standard care.

Heckman return on investment — high-quality early childhood programs deliver approximately 13% return per child per year, with aggregate returns of \$7 to \$13 (some studies up to \$17) in long-term economic benefits per dollar invested. Returns exceed standard stock-market equity returns. Source: Heckman et al., University of Chicago.

The principle — investment delayed beyond the developmental window does not produce the same returns. The returns are not deferred. They are foreclosed.

What Alberta Currently Provides

The Family Support for Children with Disabilities (FSCD) program is Alberta's principal mechanism for providing growth-mode supports to children under 18 with developmental, physical, sensory, mental, or neurological disabilities. The program is governed by the FSCD Act and the FSCD Regulation. It provides funding for respite, specialized services, aide supports, behavioural and developmental supports, medical-appointment-related costs, and other supports tailored to the child and family. It is the single most important intervention point in the province's disability architecture for the population in the developmental window.

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The program is currently failing the population it was created to serve. The evidence is published, and it is not contested by clinicians, families, or community organizations.

Inclusion Alberta, the province's principal advocacy organization for individuals with intellectual and developmental disabilities, conducted a survey of FSCD families across all 87 Alberta provincial electoral districts in October and November 2024. The survey results, published in January 2025, established the following:

Families currently wait an average of three years after applying to FSCD before having access to the full range of services the program was created to provide. The wait is structured across a three-stage process introduced in 2023: an eligibility decision, a Family Support Services agreement, and the multi-disciplinary review required for Specialized Services and Developmental and Behavioural Supports. Families in Stage 2 reported waiting an average of 10 months after receiving a positive eligibility decision before commencing Family Support Services. For families requiring only Family Support Services, the total wait from application is approximately 21 months. For families requiring the full range of supports, the total wait is approximately three years.

Fifty-eight percent of survey respondents reported negative effects from going without required supports. The reported effects included lost employment, missed developmental opportunities, physical and mental health challenges, and inability to meet the needs of other family members. Fifty-five percent of families receiving Family Support Services reported unmet needs that the program was not addressing. Thirty-nine percent of families receiving Child Focused Supports reported unmet needs.

CBC News reported in January 2025 that the FSCD backlog had reached over 7,000 children. The same reporting documented the case of a seven-year-old Edmonton boy diagnosed with autism four years prior, who had been waiting two years for speech therapy and other services with no end to the wait in sight. His father, Eddie Steele, described the experience publicly:

"It kills you as a parent because all you can do is watch. You don't have any control over the situation. And in a sense, you're kind of held hostage to a degree."

Trish Bowman, then-CEO of Inclusion Alberta, described the documented harm:

"You know that those are critical developmental years. And when you see them slipping by, you know, there's heartbreak involved. We also hear lots of families who are really going into debt and trying to figure out whatever they can do because they're so desperate not to lose those really critical years."

In March 2026, the Government of Alberta cut funding to three disability advocacy organizations twelve months earlier than the previously communicated end date. Inclusion Alberta's funding was reduced by \$500,000. The advocacy organizations that had documented the FSCD wait-time crisis, supported families through the application process, and translated community knowledge into policy input were defunded during the period of greatest need. The defunding pattern is examined in detail in Part Four. Its consequence for FSCD families is named here: the families currently navigating the three-year wait have lost the organizations that would have helped them navigate it.

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What the Wait Forecloses

The clinical evidence above and the program data above intersect in a single, unavoidable conclusion. A three-year wait for full FSCD services, for a child whose developmental window is open at age two and substantially closed by age five, forecloses the window. The Alberta government does not contest the wait time. It does not contest the developmental neuroscience. It has chosen to operate the program in a manner that allows the foreclosure to occur, repeatedly, at scale, across thousands of children currently in the queue.

For autism specifically, a child applying to FSCD at age two who waits three years for full services will receive those services beginning at age five. The peer-reviewed evidence indicates that intervention beginning before age three produces substantially greater outcomes than intervention beginning after age five. The 18-point average IQ improvement documented in early-intervention research is associated with the early window. The 4-point improvement associated with later intervention is what remains available. The 14-point difference is the documented cost of the wait.

For FASD, early diagnosis and access to services is a documented protective factor against the adult outcomes the condition is associated with when undiagnosed and unsupported. The international research, summarized by the Canada FASD Research Network and replicated in multiple Canadian peer-reviewed studies, establishes that adults with FASD who did not receive early diagnosis and intervention are over-represented in homelessness, in the criminal justice system, in mental health crisis populations, and in premature mortality statistics. A representative study of justice-involved adults in northern Canada found FASD prevalence at approximately 17.5%, with the upper confidence interval at 31.2%. A study of adult male offenders entering a federal correctional facility in Manitoba found FASD prevalence at approximately 10%. A study of Saskatchewan youth remanded for psychiatric or psychological assessment found prevalence at 23%. United States research has found lifetime incarceration rates for FASD adolescents at 32% and for FASD adults at 42%. Legal-system involvement accounts for 30 to 40 percent of the total cost of FASD in Canada, estimated at approximately CAD \$1.8 billion annually.

For cerebral palsy, motor cortex plasticity peaks in the first year of life. Rehabilitation begun in infancy produces functional outcomes that the same rehabilitation begun at age four cannot replicate. The capacity that early intervention would have produced is not recoverable through any later intervention.

For intellectual disability and global developmental delay, early intervention shapes the cognitive and adaptive capacity that determines how independently the adult can eventually live, how much of their life will require lifelong-support architecture, and how much of the adult support system will need to absorb their needs. Investment foreclosed in childhood becomes permanent expenditure in adulthood, distributed across the AISH program, the PDD program, supervised housing, group homes, family caregivers, and emergency systems.

The clinical claim and the fiscal claim converge. A province that under-funds the growth mode in childhood pays for that under-funding in every subsequent program for the rest of those children's lives. The under-funding is not a saving. It is a cost transfer — from the FSCD line item, where the investment is small and the returns are documented, to every other line item across the disability and social-services

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budget, where the costs are larger and the returns are not produced because the foundational investment was not made.

The Chronological Evidence Chain

Children with developmental disabilities who do not receive timely intervention during the developmental window do not simply remain in the same condition longer. They follow documented adult trajectories that the absence of childhood support directly contributes to. The pattern is consistent across the international literature, replicated in Canadian peer-reviewed research, and documented in the systems that absorb the consequences. The chain is the following:

Childhood without timely intervention — the developmental window closes with the capacity that intervention would have produced not developed.

School-system disengagement — the child enters formal education without the foundational skills the early intervention would have built. The school system attempts to remediate what the disability supports system was supposed to provide; the school system's resources are not designed for that role and cannot replicate it.

Adolescent mental health collapse — secondary depression, anxiety, behavioural conditions, and substance use disorders emerge in adolescence at substantially elevated rates among children with unsupported developmental disabilities. The mental illness is not separate from the original disability. It is a documented consequence of years of unsupported struggle.

Adult homelessness, incarceration, premature mortality — the FASD literature is the clearest example, but the pattern extends across developmental disability categories. Adults who were children whose developmental windows closed without intervention are over-represented in every system designed to catch the people the disability supports system failed to catch. Hospitals. Justice. Homelessness response. Emergency mental health. Mortality.

The cost is borne, ultimately, by the same provincial budget that under-funded the growth mode in the first place. It is borne at multiples of what proper FSCD investment would have cost. And it is borne in lives — in the documented homelessness, the documented incarceration, the documented suicides, the documented premature deaths — that the original investment would have prevented.

The chain is not speculative. It is documented across decades of peer-reviewed research, in Canadian-specific data, in the lived experience of every family currently navigating the FSCD waitlist, and in the population statistics of every adult system that absorbs the consequences. Alberta's current FSCD wait time is the documented entry point into that chain. Every child currently waiting is a downstream cost the province is choosing to incur, in exchange for an upstream saving that does not exist.

What the Window Looks Like

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ILLUSTRATION — A FAMILY AT THE KITCHEN TABLE

Imagine a parent at a kitchen table, holding a letter from FSCD that says their application has been received. The child is two years and four months old. The letter does not say when a caseworker will be assigned. The parent has been told, by other parents in the same position, that the wait for a caseworker is approximately ten months. After the caseworker, there is a meeting. After the meeting, there is an Assessment of Needs. After the Assessment of Needs, there is a Multi-Disciplinary Team review. After the Multi-Disciplinary Team review, there is a provincial review committee. After the provincial review committee, if the request is approved, the Specialized Services begin.

The parent does the math. Their child is two years and four months. The full range of services may begin around the child's fifth birthday. The clinical literature, which the parent has read because there has been time to read it, indicates that intervention beginning before age three produces substantially greater outcomes than intervention beginning after age five. The window the literature is describing is the window the wait list is consuming.

The parent has options. They can pay privately for the speech therapy, the occupational therapy, the behavioural support, the developmental aide, all of which are available from private providers in Alberta at rates the family cannot sustain. They can take on debt, as Inclusion Alberta has documented many families doing. They can give up paid employment to provide more of the intervention themselves at home, as fifty-eight percent of surveyed families have reported doing in some form. They can wait, and watch the window close, and hope that what their child becomes through later intervention is enough.

None of these options is what the FSCD program was created to require of them. The program was created to make these options unnecessary. The program is currently functioning as if these options are the parent's responsibility to choose between, with the public funding intended to deliver the services arriving, on average, three years after the application that triggered the eligibility for them.

The child does not know any of this. The child is in the developmental window now. The window closes at the rate that biology dictates, not at the rate the wait list moves.

What This Section Establishes

The growth mode in childhood is the most cost-effective, neurologically time-bound, and clinically consequential mode the disability support system contains. Alberta operates the principal program for delivering it — FSCD — with wait times that, on average, exceed the developmental window the program was created to honour. The harm is documented. The economic case for proper funding is established. The chronological evidence chain from unsupported childhood to costly adult outcomes is replicated across decades of international and Canadian research. The province's current direction is to maintain the

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wait list, defund the advocacy organizations that document the harm, and pay the downstream costs across every subsequent program for the rest of those children's lives.

The blueprint Part Three builds will require, at minimum, the following commitments at the FSCD level: maximum statutory wait times tied to the developmental window the science establishes; mandatory provision of essential growth-mode services within that window; binding funding floors protected from political volatility; restored advocacy organization capacity; and a governance structure that includes families and disabled adults in program oversight as a binding requirement, not a discretionary courtesy. The legislation Part Three proposes is the architecture that makes those commitments enforceable. The argument Part Four makes is that the current direction is a choice, made deliberately, with full knowledge of the documented consequences.

Section 1.4 follows the population this section examines into the next life stage: the transition to adulthood at age 18, where the FSCD eligibility ends and where the AISH and PDD programs are supposed to catch the children FSCD was supposed to support. The same wait-list and under-funding pattern documented in Section 1.3 recurs at every subsequent stage. The lifecycle frame holds because the failure pattern is consistent. Where one program fails, every subsequent program absorbs the cost.

The growth mode is the foundation. Alberta is currently failing to build it. Sections 1.4 through 1.6 examine what happens when the foundation is not there.